[redacted]

Dear [redacted]

Changes to your care – due to the national coronavirus situation

I know that this is likely to be a time of increased anxiety for you, and I want to reassure you the NHS and HPFT are working very hard to ensure our service users feel supported and that you and our staff feel safe during this uncertain time.

What this means is that we need to reduce the 'face to face' routine appointments and care coordination in line with the 'social distancing' that is now in place across the country. This means we will need to provide your care in a different way to usual until the end of June. This will include encouraging you to access on line resources for routine support.

Because of this, we have spent time reviewing everyone who is currently under our care. In doing this, a review of your care plan has identified that we can safely postpone any outpatient appointments and /or contact with a care coordinator that you have for an initial period of 12 weeks (until the end of June). A member of the team will contact you during this period to see how you are.

I want to reassure you that, if at any time you feel your mental health is deteriorating and you need our help, then you can get in touch with us and we will be there to support you, our contact numbers are:

If you need help in a mental health crisis please call our Single Point of Access (SPA)
Tel: 0300 777 0707 from 8am - 7pm or email: hpft.spa@nhs.net

Alternatively, call our 24 hr Helpline Tel: 01438 843322

For contact with the Adult Community Mental Health Services for St Albans on 01727 804674.

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[redacted]

NHS

[redacted]

It is also worth knowing that Mind Network have also launched the Hertfordshire Crisis Helpline which runs 24 hours a day and is available for anyone in Hertfordshire who wants to call to talk things through - 01923 256391.

You may also find it helpful to visit both our website www.hpft.nhs.uk and the NHS website https://www.nhs.uk/conditions/coronavirus-covid-19/ where you can find general information and information on how to stay well during this time. Our website will be regularly updated with the latest guidance, and also we will ensure we provide service updates to keep you as best informed as we can.

Some of the things to think about while you are at home to help look after your health and wellbeing are;

- stay in touch with family and friends over the phone or on social media
- try to keep yourself busy – you could try activities like cooking, reading, online learning and watching films
- do light exercise at home, or outside once a day
- For more advice www.nhs.uk/oneyou/every-mind-matters/coronavirus-covid-19-staying-at-home-tips

I am very sorry for any inconvenience caused by the need to change your appointments with us. I do want to stress that if you feel your mental health is deteriorating and you need support we are here for you and please do not hesitate to contact us.

If you have been advised by your GP or other NHS service to follow 'shielding' guidance in relation to other healthcare conditions you may have, please follow that guidance and let us know so we can take that into account in the provision of your mental health care going forward.

In the meantime, please do make use the online resources available to support your health and wellbeing and we will contact you again in a few weeks time to update you further.

Best wishes

[redacted]

as one

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Dear

Next Appointment:
No Medical Appointment is booked, however, remains open to the secondary mental health service for psychology assessment

Summary of Appointment

I had a virtual consultation with Mr Jamie Davies and his care coordinator, Hannah Mendoza-Wolfson, on 9th July 2020. This review was arranged at the request of Jamie who informed Hannah that he was very concerned with the new diagnosis. When we started the consultation, Jamie told us that he just had five to ten minutes as he was in the middle of a survey. We offered him to reschedule the consultation to have more time with him, but he declined.

At the consultation, Jamie told us that he is trying the breathing techniques that Hannah suggested to him and they can be useful but not all the time. He said that we need to arrange another meeting but this time we should invite his father and his brother. He said that it is important that we need to consider wider picture and involving his ex-wife would be very useful. He talked about being very stressed about not having access to his daughter.

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He said we need to give him clarity about the situation that surrounds her daughter so that he can make a reasoned and informed decision as to whether to make peace with his own past. He thought that this was an urgent matter despite not seeing her daughter for years. Jamie also said that "It's too late, what I want is a solution-based approach". He repeated the theme of us as a service give him clarity around the situation that surrounds his daughter so he can make reasoned and informed decisions on how to proceed in my life and then all of us can breathe. He said "I felt pressured by you, I felt pressured by my work; I need to see some actual result". I reminded him that we don't have the power to bring his daughter back into his life and this was something that discussed at the meeting we had with his mother and his partner.

I tried to explain to him that this meeting was arranged at his request because he wanted to discuss the possible diagnosis of complex PTSD. He then said that he can read about it on the internet. He told us that he needed to go and we ended the consultation. He is aware that he is on the waiting list for psychology.

I understood from Hannah that he got a job as a part time surveyor recently, but the company has asked him to do five days instead of three days and this is something that he is negotiating with them.

Impression

[redacted] presents with anxiety and negative self-concept; he has significant worries about day-to-day activities, and has occasional episodes of heightened anxiety characterised by aggressive and demanding behaviour and/or quasi-psychotic symptoms. He has no biological symptoms of depressions.

[redacted] and I do not think that Jamie has paranoid schizophrenia. We noted that there were times when he expressed paranoid ideations, was perplexed and his speech was difficult to understand. However, we believe that these symptoms can also be explained in the context of the difficulties that he experienced in his life.

Our understanding is that Jamie might have had experienced adverse childhood events which made him vulnerable to mental health problems following the difficult separation and divorce he experienced. I believe that the divorce was the trigger for Jamie to lose his previous level of functioning. It is clear that his mental health problems have become significant since his divorce around ten years ago and have impacted his work life and relationships with his partner and family. He is abstinent from alcohol and other illicit substances, and reports no active suicidal ideations. He has ongoing psychosocial stressors involving his ex-wife and lack of contact with his daughter.

Despite being very resourceful, Jamie continues to struggle to come to terms with the divorce and continues to be stressed about not being able to have access to his daughter.

We also noted that Jamie's behaviour can be challenging and demanding towards professionals that are involved in his care.

Care Plan Agreed with Patient

Mental Health Medication
None (stopped risperidone in October 2019, then fluoxetine in December 2019)

Physical Health Medication
Nil known

Recovery Goals/Interventions (including lifestyle, employment and accommodation Plans)

- We feel that [redacted] will benefit from a psychological assessment to explore his early life experiences. I believe that if he does not address his psychological issues, it's going to be difficult for him to function up to his expectations as an adult and to remain stable in his mental state. I have referred him for psychology. He had previously been referred and accepted to psychology whilst under the Welwyn Garden City team, but this was never started due to an admission.
- In the meantime, Hannah agreed to work with him towards discharge and to complete the grounding work she started with him offer him to help him to manage his overwhelming emotions and worries.
- Hannah to liaise with [redacted] in order to find out when is she planning to offer psychological assessment to Jamie.
- With [redacted] consent, Hannah to contact Jamie's mother and partner to update them on the care plan.
- [redacted] is aware that I am not planning to offer him another outpatient appointment. As he is not on any medication for his mental health, he has not been offered physical health assessment by us.
- [redacted] is very resourceful and is very good at seeking help in his own way. Jamie agreed to contact us, mental help helpline numbers as above or his GP if he notices deterioration in his mental health.
- Prior to the lockdown, Jamie was engaging in various activities including Tai Chi, running, AA, attending one Mind group a day, and salsa dancing. He also writes poetry, does origami, and is currently accessing music therapy. He has been working with a vocational officer from Mind, on the waiting list for therapy.

Actions for GP

None

Yours sincerely,

Electronically checked & approved by Doctor

[redacted]

[redacted]

[redacted]

[redacted]

Although you are currently still open to the team, our work together has now come to an end, so I wanted to write to you to summarise our journey. We started work on 18th October 2019 after your transfer of care from Welwyn City Adult Community Mental Health Service, and we met on an almost weekly basis until 6th August 2020 . During this time, I have seen a change in your presentation and the way that you think.

Initially the plan was to do a brief intervention piece of work for your anxiety, as you were experiencing some big changes in your life, including leaving your job. You were at times so anxious that you would vomit; however I note that this has now stopped which I am glad about.

During our sessions I introduced you to CBT, and we looked at how thoughts, feelings and actions are connected; by changing one we can likely change the other aspects of the 'hot cross bun'. We spent some time looking and practicing thought challenging, as well as considering the emotions negative thoughts would evoke - in particular anxiety and anger. Although you were able to understand the concept, you sometimes found it difficult to come up with an alternative thought.

You worked hard completing the tasks and homework that was set, including noticing positive events in your life, and negative thinking styles and how to challenge that thought. You have worked hard to identify stressors in your life and consider ways to reduce the stress; you discovered that origami and Tai-Chi were helpful for both distraction and relaxation purposes, and through MIND developed an interest in writing. Together, I taught you some anxiety management strategies including a breathing technique (in for 5 and out for 7), as well as paired muscle relaxation (squeezing a muscle group as you inhale and relaxing it when you let your breath go). I also explained the benefits of intense exercise (quick burst of physical activity), or putting your

face in cold water/ an ice pack across your eyes and bridge of your nose, if you felt particularly overwhelmed with emotion.
You have been resourceful in seeking help. Initially you would call the mental health help line on a regular basis, often just wanting to chat and be given reassurance for your anxiety. Together we looked at reasons that you felt compelled to call them, and later on in our work call me, and you noted that when you are anxious you find it difficult to think things through clearly and it helps to have other people's opinions. You also told me that you often turn to Lion Heart, who at one point had to put restrictions on your calls in place, as well as MIND and the Samaritans. During lock down you also sourced a private music therapist who you found helpful. We worked on putting a list of strategies you could implement to reduce your anxiety first, and then a list of other people who you could turn to for support. You reflected that your mum, dad and Nagis are all helpful people at times, as is your AA sponsor with whom you have a good relationship.

During our work you have had to put yourself in potential awkward or stressful situations, including job interviews and going to Bangladesh for the first time. You were able to manage your anxiety and alter your thoughts towards these experiences, and you found that both have been positive- you really enjoyed your trip at the start of the year and are now in full time employment.

I appreciate that at times your new job is stressful. This for a short while led to an increase in your calls and texts to me. However you did well to reduce these calls, and we considered other strategies for your anxiety, such as visualisation. Together we practiced visualising yourself in a safe and comforting place, imaging yourself being there using all of your senses. We also practiced visualising a potentially difficult future scenario and how you could cope in it, imagining yourself walking through all the steps. With our practice scenario you were able to problem solve the various potential obstacles (buy dinner using cash so that you would get change and have cash for a parking meter in case it was needed the next day), which shows how far you have come in being able to take different perspectives and think through different angles of a situation.

Since moving to our team you have had your diagnosis changed from Paranoid Schizophrenia to Anxiety and potential complex PTSD. Although you had not agreed with the initial diagnosis, having it altered appeared to throw you a bit and perhaps impacted on how you saw yourself. I'd like to remind you that you are more than a diagnosis, and they are there to help inform a care plan and consider how we can best help you. As you know, we have therefore put you forward for a psychology assessment. In our last call you stated that you do not think you have a mental health problem and that everyone has ups and downs and goes through difficult things in life; whilst I agree that life often presents difficult situations, it is important to remember that you have a support network to help you through these times. Moreover, I know working on improving and maintaining your mental health is important to

[redacted]

you, and you have worked hard over the years to do this; please remember that helping yourself may not be by using lots of services, and feeling obliged to attend a group or talk, but rather making sure you carve out time to do things that give you a sense of enjoyment, fulfilment or relaxation.

Going forward:

You have a psychology assessment with Dr. Bains in September
Should you feel the need to talk to our team you can contact duty on 01727 804674, or the Mental Health Help Line on 01438 843322
You also receive support from AA, Lion Heart, MIND and Samaritans

I know that you have already written some of these strategies down, but as discussed, I would recommend writing a plan that includes which strategies to use and when for when you feel overwhelmed, stressed or anxious, so that you know what steps to follow to initially help yourself. I have included a plan for you to fill in if you wish. It has been a pleasure getting to know you, and I wish you the best of luck for the future. You have worked hard, and I am sure will continue to do so.

As a service we appreciate all feedback, so that we can continue to work towards always providing the best service possible. If you would like to leave feedback, you can do so here: https://www.hpft.nhs.uk/service-users/having-your-say/

Yours Sincerely,

[redacted]

NHS

I met with [redacted] on 18th September following referral by Dr Elvan Akyuz, Consultant Psychiatrist, for assessment for suitability for psychological therapy. The meeting was conducted by video call due to Covid 19 restrictions.

Interview with Jamie

At the start of the meeting, [redacted] seemed very preoccupied with the screen on the video call, highlighting that one of the windows had the caption "patient" which he thought described a power imbalance between us and made him uncomfortable. He was tangential throughout the assessment meeting and returned to talking about the screen at various points in our meeting. It was very difficult to get any sense of any of [redacted] current difficulties beyond his practical needs. He began by telling me that his freezer and car were both broken and would not be fixed until the following week which was causing him concern. He also spoke about the cost of these repairs.

He told me a little of his current relationship with his girlfriend, _____, who lives in East London and told me that he lived alone. When asked about his thoughts about psychotherapy, [redacted] stated that he was pleased to be having a psychotherapy assessment because he wanted answers about why he was taken away from the family home by the police. He became somewhat irritable when I tried to ask more about this event, stating that he wanted understanding as a human not from a

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therapist and that he wanted an amendment in the police records about him. He also went on to state that "it shouldn't happen to children, their parents shouldn't split up".

He then disclosed that he had had no contact with his daughter from his marriage who was now 12 years old and that this allows him to garner sympathy from others "poor me". He spoke about having had previous therapy for alcohol addiction. Jamie then went on to tell me about how his own father left when he was aged 12. He recalled being in a very anxious state and stopped eating. He remembered his grandparents coming to his school and giving him sandwiches to eat in their car. Jamie attended his local primary school until the age of 13 and then attended Haileybury School as a boarder. He described this as a difficult experience as there were" 50 boys in a dorm" and spoke of "terrible abuse" but did not give any specific details about this.

Impression

It was hard to get a clear narrative to the events in [redacted] past from his account due to his being tangential and at times obfuscating in his speech and it being difficult to make any emotional connection with him. For example he initially told me that he was sexually abused by his mother but when asked for more details about this, he told me that it was not true and he had just said it but he wasn't sure why. His account of his background was therefore fragmented with a lack of details or cohesion. In addition, my sense was that this fragmentation seemed to be more pronounced when I attempted to reflect with him about anything potentially painful, such as his father leaving the family. He would at these times become either irritable or further fragmented. It was also difficult to establish what other support he is currently getting in the community (as he did not answer these questions), although I understand he is seeing MIND and a Music Therapist.

I understand that the previous diagnoses of Psychotic illness and ASD have been considered for [redacted] and that a current diagnosis of Complex PTSD is being considered. I could find nothing to support this at my assessment this and there were no experiences related to past trauma which seemed accessible to him although he did allude to negative experiences at boarding school but again it was not clear.

Recommendations

It is my impression that [redacted] would not be able to make use of or benefit from psychotherapy currently due to his limited reflective capacity and ability to form a cohesive narrative. He does seem to appreciate supportive contact and may benefit from supportive interventions which I believe he accesses currently in the community.

[redacted]

COPY

2

[redacted]

Patient's Reported Condition

headache, very tired, very anxious which is his main concern

Pathways Disposition

To contact a local service within 24 hours (Dx08)

Selected service: Mental Health Single Point of Access

Consultation Summary

Illness
Warm to touch
Behaviour change
No threat of violence
Problem duration, more than 2 weeks
Divorce/relationship breakdown in last 6 months
Behaviour or mood disturbance, normal life impossible

Pathways Assessment

An injury or health problem was the reason for the contact.
Heavy bleeding had not occurred in the previous 2 hours.
An illness or health problem was the main problem.
User Comments: headache, very tired, very anxious which is his main concern
The individual was not fighting for breath.
The main reason for the assessment was not an allergic reaction, heart attack, chest/upper back pain, probable stroke, recent fit/seizure or suicide attempt.
The main reason for contact was not new confusion, declared diabetic hypo/hyperglycaemia, or ICD shock.
The skin on the torso felt normal, warm or hot.
Pathway selected - Mental Health Problems
There was no worsening of a known mental health problem.

[redacted]

The individual had been behaving abnormally.
The individual had not attacked, threatened or intended to harm anyone.
There was no ingestion/inhalation of something harmful in the last 72 hours.
There had been no new onset cough.
There had been no raised temperature within the last 2 weeks.
There was no new loss of taste or smell in the last two weeks.
The duration of the problem was more than 2 weeks.
The individual denied suicidal thoughts.
Withdrawing from alcohol was not reported.
There was no daily headache on waking.
Medication type or dosage had not changed within the previous 2 weeks.
The individual was not repeatedly falling asleep.
There was a divorce or relationship breakdown in the last 6 months.
The problem was so severe that normal life was impossible.
The individual had not considered harming themselves or anyone else.
No one else was considered to be vulnerable and at risk
Instructions given were: The individual needs to contact a local service within 24 hours.
Directory of Services referral: Mental Health Single Point of Access
Advice given: Worsening

Advice Given

If there are any new symptoms, or if the condition gets worse, changes or you have any other concerns, call us back.

[redacted]

HCNS

Hospital & Community Navigation Service GP Feedback Form

[redacted]

You recently made a referral to our service for the above patient to support with:

Physical health and wellbeing	
Mental health and wellbeing (including emotional support)	Y
Advice about finance or benefits	
Social isolation or loneliness	
Accessing community groups and activities	
The home environment and/or home safety	
Staying independent	
Carer support	
Paperwork and/or form filling	
Other	

We are now working with this service user to meet agreed outcomes below	
We have worked with the service user and have achieved the outcomes below	
This was an inappropriate referral for our service (see below for details)	
Patient has not engaged please contact to see if help still required	
Patient has not engaged with our service (see below for details)	Y

Agreed Outcomes: - Patient referred to SP due to debt issues.
Achieved Outcomes: - Patient declined all help that was advised.
Reason for inappropriate referral:
Reason Patient has not engaged, and actions taken: -Contacted client, patient was referred for debt related issues, but states.

Delivered in Partnership by:

Watford Community Housing | Carers in Hertfordshire | Hertfordshire age | Watford & District Citizens Advice Bureau | British Red Cross | POWER

[redacted]

HertsHelp

HCNS

that he has no debts or finance problems. Referral also stated that patient suffers. with MH and has been discharged.

I asked what he would like support with, patient stated that he is worried about, upsetting his girlfriend as he does not want to be with her and wants to be with someone else. He was asking for help in choosing which one. I stated to patient. that I cannot support him with this issue this is a decision that he needs to make himself. He stated that he is nervous to tell her and very worried that she will call. the police. I asked patient why he thought this, he said because I may hurt her feelings.

I asked if he wanted to have another referral made to MH, but he has declined this. He stated that is not the problem, the problem is with his relationships. He stated that he has paranoia, but again has declined help.

Patient stated he still has contact with his old MH social worker and stated if he wants support he will make contact with them. I asked if he required any other support but he stated no case is closing.

GP's - For any matters relating to this case please contact hertshelp@nhs.net or call 0300 123 4044

Date Typed: 06/02/2021
Date Sent: 10/02/2021

Impression: Low mood
Medications: NIL
Lead Professional:

Source of referral: Self-referral
Date and time referred to EMHAH: 06.2.2021, 12:35
Date and time assessed by EMHAH: 06.2.2021, 13:00

'Due to the current COVID-19 crisis, all assessments completed are currently briefer and focussed on the current circumstance and risk.'

Reason for referral and current situation:
elf-presented to the Luton Mental Health Assessment Hub early this afternoon. onfirmed to me that he drove to Luton and Dunstable Hospital because he was feeling 'mentally unwell.' said 'there is no hospital in Harpenden or St Alban's that's why I came here.' He also reported he called the mental health crisis line, but could not remember the time he rang and what advice was given to him. He said a lady from the reception at L&D advised him to come to the Luton Mental Health Hub.
has confirmed his address: AL5 5EW. Giving that his address contacted the Hertfordshire Menta for mental health support. I spoke to the Referral Advisor (Debbie). She clarified that James is under the CHMT in Hertfordshire, and advised to contact her back after talking to James.

I spoke to d he told me that he thinks he is not mentally well. He reported he is currently in a relationship with an African lady who has been taking quite a bit of money from him. He feels the lady is merely using and playing him, but does not love him. He said he does not know the lady really well even though she wants o move to her place.
owed being in another relationship with a lady from Bangladesh whom he said he likes very much because she is quite 'independent. However, James admitted it has been difficult for him to contact his ex-wife who lives in Northampton with his 13 years old daughter. He asserted that he has been missing his daughter due to lockdown and ex-wife not answering the phone, this makes him worried.
maintained that he is worried because he has spoken to his colleagues from work about his ex-wife and because two of his colleagues are from Manchester where his ex-wife is from, they will reveal what he has told them to her.
At the assessment, got off the chair and stood by the window. He took his jumper off as he said he was hot and was looking outside the window. He then said he's feeling

'resentful' and did not want to continue talking. He denied suicidal ideation and thoughts to harm self and others.

Family/Relatives views
No family/relatives present at the assessment

Drugs/Alcohol/Smoking:
[redacted]eported he has stopped drinking and smoking. Denies use of drugs

Abuse: None reported

Forensic: None reported

Social History:
lives alone in a 2 bedroom flat which he owns. He is a surveyor and currently works from home due to covid-19.

Past Psychiatric History
[redacted]has a diagnosis of depression and is prescribed fluoxetine 40mg. He is known to the CMHT in Hertfordshire.

Physical Health: None reported

Mental State Examination:
Mood: Subjectively, [redacted]escribed feeling 'mentally unwell' and said he was 'resentful' that he could not tell me other 'stuff.' Objectively, he appeared confused and not willing to talk about his current situation.
Appearance: Appeared well-kempt. No evidence of self-neglect.
Behaviour/Speech: [redacted]as somewhat co-operative and made limited eye contact during the assessment. Rapport was established with difficulty. His speech was spontaneous, and appropriate; normal in tone and volume.
Sleep/Appetite: Reported poor sleep/appetite
Perceptions/Concentration: There was no paranoid ideation and no perceptual abnormality
Cognition: Grossly intact
Thoughts: No evidence of thoughts insertion/broadcast
Capacity/Insight: He seemed to have good insight into his illness and is capable of making informed decisions. He displayed the capacity to contact the local Mental Health Helpline – number provided.
Risk Assessment: Denied any suicidal ideations or plan. No thoughts to harm self or others.
Safeguarding: None.

Impression
[redacted] is a 45 year old Caucasian man who self-referred to the LT EMHAH. He appeared confused and worried about being in relationships with two women whom he believes are with him just because of his money, but they don't love him. He finds it hard to cope with the fact that his ex-wife is not answering his phone calls and he is not able to see his daughter. The impression was that [redacted] became indignant as he was talking about his relationship problems; hence he refused to be asked further questions. He denies any thoughts of suicide and self-harm. He needs to continue working with local CMHT to support him.

[redacted] [redacted]

[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

DISCHARGE FROM SERVICE

Following your re-referral to mental health services, you were offered an initial assessment with me on Friday 5 March. You called up to cancel this appointment and I spoke to you about your choice and reason for referral.

You told me that you are not interested in medication and are having private therapy elsewhere so did not think that mental health services could offer you anything additional.

You said that you continue to be under AA and has contact with Mind, as well as your private therapist. You expressed no thoughts or plans to harm yourself or others, although you did talk about wanting to get hold of you estranged wife (who you said is not your legal wife).

There was quite a lot of inconsistency in the content of what you said but you did get across that you want 'justice' in relation to contact with your daughter. You were aware that this was beyond the remit of mental health services and you are aware of the Citizens Advice Bureau and Families Need Fathers.

We ended the call there without completing the Initial Assessment because you said you had referred yourself when you were 'in a panic' last month and do not think you really want our service. Following discussion with the rest of the team, there was no evidence that your mental health was markedly different from when you were discharged from the service last year. You care co-ordinator at the time, Hannah, has written to you previously outlining the work you did together, and the support you had under our service.

We would encourage you to continue using skills you learnt through those sessions, and the support you have in the community.

as one

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If your situation changes and you feel you need to see one of our professionals please contact your GP or you can self-refer to SPA.

Single Point of Access (SPA) Tel: 0800 644 4101
Email: hpft.spa@nhs.net.

For out of hours advice and telephone support please call the Mental Health Helpline on 0800 644 4101.

Yours sincerely

[redacted]

Copy to :
GP for information :

[redacted]

[redacted]

D

as One

[redacted]

[redacted]

[redacted]

Date of Birth: 30/07/1975
NHS Number: 428 422 6908

We recently received a referral for you and have successfully contacted you by telephone on 17th May to discuss this in more detail, to identify how best we could support you.

Following this conversation and after careful consideration the team has established that you have support from your private therapist, AA, MIND and your GP and that it appears that you have the support that you need at present and we won't be offering anything from our Team at this point.

However, in my email to you I have sent information regarding Mindfulness exercises and a Wellbeing Self Help Pack as agreed when we talked.

I also provided information of two organisations that you can self-refer to if and when required. These are:

1. **Herts help including community navigators**
   https://directory.hertfordshire.gov.uk/Services/7823

About : Helping people connect and reconnect with like-minded people and groups in the community. The Community Navigator works with the individual and establishes the relationships with all of the voluntary sector organisations and activities which will support the

person with the issues they face; e.g. they may be a family carer, could be in debt, or are isolated; and need network of community support to ensure they stay well, independent and feel supported by their community.

2. **Complex needs service https://www.hyh.org.uk/our-services/complex-needs-team**

About: They provide practical support that is built around you and your individual needs, promoting personal independence and recovery.
They can support you with:

- Emotional and practical support
- Solving real-life problems, including housing, debts and benefit issues
- Signposting to carer's services and support groups
- Access or onward referral to other relevant community based services
- Informal family support
- Reducing episodes of crisis
- Building confidence, self-esteem, social networks and life skills
- Building new social networks

I have also been made aware of an organisation called Mens Sheds https://menssheds.org.uk/ that may be of interest to you and have also enclosed a leaflet with organisations to call for support.

I am writing to inform you that you will now be discharged from secondary care to the care of your GP.

If your situation changes and you feel you need to see one of our professionals please contact your GP or you can self-refer to SPA.

[redacted]

For out of hours advice and telephone support please call the Mental Health Helpline on 01438843322.

[redacted]

[redacted]

Time of Assessment: 16:35

Source of Referral:

presented to the A&E department complaining of low mood. The triage motes said *"Having low mood leading up father's day as he has not seen his child for 13 years, also has work stress and PT also reporting its worse with the pandemic on his mental health".* He was referred to PLS for assessment.

Current Situation:

was sitting in cubicle 18 and was talking on the phone. I introduced myself to him and also introduced him to the student nurse. He continue talking on the phone with his friend saying, "*I do not feel well mentally, I might feel better after I speak to these people.*" He continue speaking to his friend and looked at us and made no attempt to end the call. My colleague and I waited for him to finish speaking and his friend told him to contact him when he finish speaking to us. continued his call for a while them ended this. I introduced myself and the student to him again.

reported that he drove to A&E as he was sitting in his flat feeling sorry for himself. He explained that he has work issued and relationship issues. He said that its father day tomorrow and he has not seen his daughter for 13 years. He told me that his daughter is 13 years old. He recognised that sitting at home feeling sorry for himself was not positive and said that he wanted to talk to someone. Reassurance was given and I reminded him that I had seen him 2 years ago and was aware of some of his social stressors he had experienced. reported that he is in a relationship and he had an Islamic wedding. He said that his wife wanted a baby and they could not have one and didn't give an explanation of this. He said that he didn't have a good relationship with his father and only saw his father at the wedding and want to proof to his father that he is better than what he thinks of him.

said that he work is stressful and when I asked for details he said that I was asking too much questions and wanted to know why I wanted the information. I explained to him that I wanted to assess how this is affecting his wellbeing and he appeared to be hesitant in

giving me some information. [redacted] said that he gets angry when people said that he has mental illness, he feels it's an offence. He sees it as a lack of knowledge and said that he has no knowledge of where is child or his ex-wife is. He then proceeded to say that he wanted me to find them, wanted me to contact the police, and asked them to look for his child on his behalf. I explained to him what the A&E department is for and told him about social services and there role. He said that he had a social worker who told him that she want able to get involved in finding his daughter and also he was told to go the A&E as we could help. I explained to him again the role of emergency services and advised that he seek advised through lawyers and the courts with regards to his daughter and we weren't able to do this in the hospital [redacted] denied any current suicidal thoughts plans or intent and made it clear that the only reason he came to the department was to talk about his relationship issues and for him to get help to find his daughter. He did not feel that he has a mental health problem.

Past Psychiatry history:
[redacted] as been seen and assessed in the A&E department previously and was also seen at the mental health hub in February 2021. He was discharged to the community services. He has a diagnosis of depression and is taking antidepressant. He told me that he is on antidepressant and was not able to remember the medication he is taking.

Personal and Social history:
I did not take a full history from [redacted] s this is already documented on his RIO notes. He lives on his own and he is in full time work, working as a surveyor.
Forensic History: None disclosed.

Drugs and Alcohol:
[redacted] nied any alcohol intake and told me that he has been sober for 10 years. He denied experiencing with any illicit substances.

Mental state assessment:
[redacted] as causally dress. He maintain good eye contact and good rapport was established. He however at times was reluctant to give some information. He appeared confused at times and would be selective in information that he wanted to share.
Speech: Speech was clear, tone, volume and rate was ok
Mood: [redacted] described his mood, as *"I feel safe in going home"*. He presented at irritable at times due to the questions that I was asking him. (o) euthymic.
Thoughts: He denied any thoughts of wanting to harm himself and he did not express any plans or intent. There was no evidence of thought disorder and he was orientated to time, place and person. He denied any hallucinations and he has the ability to understand and retain information.
Sleep: Reported that he sleep for 7 hours.
Appetite: Said that had a sandwich to eat earlier.

Carers/Families views: N/A

Risk assessment:
Self- Low.
Others- Low risk.
Violence- None disclosed.
Safeguarding Concerns Adult/Child- None.

Impression:
45 years old gentleman presented to the A&E department reporting low mood particularly that its father day tomorrow and he isn't able to see his 13 years old daughter. He has a diagnosis of depression and has been taking his prescribed medication. On assessment, he told me that he was sitting at home and was feeling sorry for himself and felt it was a good

[redacted]

[redacted]

idea to present to A&E to speak to someone. He was coherent and insightful and clearly denied any thoughts of wanting to harm himself. He has been experiencing social stressors at work and not been able to see his 13 years old daughter which an ongoing issue. I do not feel he is a significant risk to himself and others and his current symptom can be managed by his GP. I explained to him that he will need to seek legal advised reading his daughter and advised that he also self refer to total wellbeing.

Plan:

- Discharge back to GP and advised to self-refer to total wellbeing.
- He will continue to be compliant with medication.
- I will advised the GP of his attendance to the A&E department and send copy of assessment.
- He is aware of emergency services available and will contact them if his mental health worsen.
- Discharge from PLS

[redacted]

[redacted]

Date of admission to L&D: 20/06/2021
Date referred to Liaison Psychiatry: 20/06/2021
Date assessed by Psychiatric Liaison: 20/06/21
Time seen: 12:00 and 13:00hrs

Reason for referral:
has been referred to the rom the A&E department and according to the triage notes the reason for referral is "*patient was assessed yesterday by MH team. Coming back today because wants his family back and believes NHS can help*".

Current presenting problems:
I attended the A&E department and met with James in the waiting area. I introduced myself and my role and he agreed to engage in mental health assessment. He was escorted to cubicle for the purpose of privacy and dignity.

The first thing James said to me was "*how long is this going to take*". I explained to him that I am unsure as he has presented to the department asking to speak with Psych. He then said "*well I will tell you what I have to say and then I will leave*". He went on to say that he attended yesterday and see a lady and man "*and I asked them if they can help to get my family back, they left the room and didn't return so I have come back to see if they can help me*". I explained to him that I am aware he was seen and according to the notes my colleague completed a full assessment with him. They had explained that they are not able to help with him locating his ex-wife and 13 year-old daughter and the plan was to discharge back to GP and for him to refer to total wellbeing. However, James was very dismissive of this and was adamant that my colleague told him she could help him was asked about his current thoughts and he told me "*you will have to rent or buy them*" he then started to cry and laugh at the same time.

[redacted] then said "*I want to get the self respect back within the family*". I asked him what he meant by this and he said "*I always have relationship with the NHS and not all relationships are perfect, I'm happy that you can re-direct me back to my family but the thing is I don't know where they are*". I proceeded to explain to [redacted] that I have no power to make contact with his ex-wife and daughter who he has had no contact for the past 13 years, at this point [redacted] then said "*I don't want to be the joker anymore*". I then asked [redacted] if he has ever been admitted to psychiatric ward before and he said yes he was admitted to Kingfisher court last year and said "*it was to improve my mental health*". I then asked him he feels he is mentally unwell at present time and he sad "*no I'm ok, sometimes things aren't right, I don't want to play the joker anymore do you understand me*". James was then observed to break down in tears and then hysterical laughter saying over and over "*I don't want to be the joker*". He then became agitated and restless and told me "*you have the powers to help get my family back*". He then said, "*I hope I'm not shouting at you, am I shouting at you, I don't think I am*". At this point I left he room as James appeared to be unpredictable and he moved closer to the sofa and stared at me with a fixated look.

I returned to the department half an hour later with my colleague, Peace, and we spoke with [redacted] I Asked him if he could remember what I had asked him earlier and he said no. He went on to ask us if we could help him locate his family and then he would answer the question "*oh no I suppose you can't*". I asked [redacted] about calling the single point of access and he said he was aware he had called them. He also told us that he had called the Samaritans "*now they have blocked my number because I was calling them to Many times*". The more we engaged with James he became restless and agitated and towards the end he was pacing the room. James told us that the NHS is to blame for his family being taken away and the doctor is to blame for his "*depression*". He continuously told us "*I do not want to be the joker anymore*". When asked if he feels he maybe relapsing in his mental health de declined stating "*no I'm not I need you to help me get my family back, you have the powers, you can check your records and find my daughter*". When he was told that this against NHS policy to search other peoples records he then said "*you are a very powerful organisation and have the powers to do it, you guys played your part in taking them away so today you are the voice of the NHS*".

Past Psychiatry/Medical history.

I contacted Hertfordshire Crisis Team and spoke with CPN, [redacted], who informed me that James has a history of mental illness dating back to 2011 where he has multiply psychotic episodes that have resulted in psychiatric admission and the last one was in 2018, where he was assessed under the MHAA, and detained on a section 2 MHAA and admitted to Albany lodge. Jackie informs that James has a history of violence and aggression and it is recorded that he pushed his mother and head-butted his step father in the past. Jackie informed that James had initial assessment on the 28/05/2021 and the plan was as follows:

Following this conversation and after careful consideration the team has established that you have support from your private therapist, AA, MIND and your GP and that it appears that you have the support that you need at present and we won't be offering anything from our Team at this point.

[redacted] so informed since 22/5/2021 – 19/06/21 [redacted] has called the single point of access 46 times and has been asking them to help with locating his family. More recently t/c received from [redacted] He remembered he spoke to me yesterday and then the call was transferred to my colleague. He said he presented at A&E yesterday – started to describe the time there. Taken to a room, there was a spanner and nail behind the sofa and two

[redacted]

nurses came to see him and then left. He appeared subdued and non coherent in speech. He would talk ask questions and answer himself. Anything I said he would tell me not to suggest or advice. He was bitter and angry at times then would calm. He spoke about psychosocial matters. This is more than mental health. Difficult conversation, trying to make sense. He eventually spoke about his daughter and wife.

- "I was so drunk I lost my daughter and wife".
- "Can you help me bring them back".
- "I was told by your organisation I won't get better unless you give them back to me".
- "You and the police told me to go away from them".
- "I'm not going to get better unless you tell them I need them back".
- "I don't want you to say, I want you to do action".
- "You're the one that asked me how can I help, so bring back my daughter".
- "Is there a way to signpost me back to my family".

Ample time was given to support, however, whatever I said he didn't like. Some sarcasm and rude comments. I informed him that I will terminate the call if he continues.

Mental State Examination/Capacity & Consent:

At time of assessment I feel that James did not have capacity to make rational, informed decisions and did not understand about his care and treatment. He was unable to retain and relay information given. His memory appeared slightly poor for recent and remote events, i.e. he was able to recall the events leading to his recent referral to Psychiatric liaison services. He was well oriented to place, time and person. His concentration and attention were both poor as he was fixated on his ex-wife and 13-year-old daughter.

[redacted] was observed to be well kempt and dressed in his own clothing. He was pleasant and polite however, he became easily agitated and restless. He appeared to be anxious as he would get up and pace around the room. There was no aggression observed during this assessment. He held limited eye contact and at times would stare at me for a while. His concentration was poor as he was preoccupied with his ex-wife and daughter. No poverty or pressure of speech however was irrational and off topic at times query flight of ideas. [redacted] presented as preoccupied, distracted and would take a long to answer questions. He was suspicious and guarded especially when answering questions around his mental health. He would answer with a question that was not relevant to topic. He denied having any self-harm or suicidal thoughts however told us that he has an addiction to "*sex and love, technology, coffee nicotine and alcohol despite not drinking for 10 years*". James believes that the NHS has super powers to locate his ex-wife and daughter and bring them back to him despite staff telling him otherwise. I feel he lacks insight into his mental illness and he does believe he has mental health problems.

[redacted] described his mood as being "*stable but then what is that because I'm stressed*" at present time stating that on a scale of 1-10: 1 being very low and 10 being very happy, James rated his mood as being 5/10. Objectively: anxious and labile in mood [redacted] denies feelings of worthlessness and hopeless. It was difficult to engage with [redacted] informs me that he is having very poor sleep but then said he sleeps 6 hours per night. When asked about his appetite he said, "*I have a huge appetite and that's to get my ex-wife and daughter back*". He told us that he goes running which helps.

[redacted]

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Risk assessment:

Self: [redacted] has self-presented to the A&E department on two consecutive days asking staff to help him locate his ex-wife and daughter who he has not seen for 13 years. He presented as if he is relapsing in his mental state and he is at further risk of deteriorating if not treated which is triggered by current stressors.

Others: [redacted] denies having any thoughts of harming others however, [redacted] informed that he has a history of aggression and violence and can be very impulsive and unpredictable.

Violence/Forensic: None at present time.

Carers/Families views:

I have spoken with NOK, [redacted] mother [redacted] with his consent. She told me that he is constantly calling asking for her help. He has refused and she attend the department "*as he doesn't want to be mollycoddled*". She says that it all started when he and his wife divorced and she took his daughter away, mother says it was a messy divorce. She says that he does suffer with anxiety in stressful situations. She says that he has recently broken up with his girlfriend who he dated for a few years and she was supportive of him. I explained to her about the MHAA and she said she understood however would not want him to be admitted to psych ward.

Impression:

[redacted] is 45-year-old gentleman with a long history of mental health dating back to 2011 where he has had multiply psychotic episodes and admissions to a psych ward and the last admission was in 2018 where he was detained on a section 2 MHAA. James has self presented to the A&E department on 2 consecutive days asking staff to help him locate his ex-wife and daughter who he hasn't seen for 13 years. He presented as if he is relapsing in his mental state and he is at further risk of deteriorating if not treated which is triggered by current stressors.

Management plan:

- Discussed with SPR on call DR Lunnawatt and he agrees for referral to EDT for MHAA
- Referral to be made to EDT for urgent assessment
- Sister [redacted] in the A&E department has been informed of the above plan, and informed if James leaves the department police and security to be called
- [redacted] was informed of the plan to refer his for a mental health act assessment.
- [redacted] was seen and assessed by EDT and not detained with no follow up and to continue with private therapy
- Discharged from PLS

[redacted]

The Luton and Dunstable University Hospital Emergency Department

Discharge Summary

Telephone No.: [redacted]

PATIENT DEMOGRAPHICS	GP PRACTICE
Patient Name: [redacted]	DAVENPORT HOUSE BOWERS WAY, HARPENDEN HERTFORDSHIRE, AL5 4HX
Date of birth:	
Gender:	ATTENDANCE DETAILS
NHS number:	time of arrival: 19 Jun 2021, 15:35
Other identifier:	
Patient address	
	REFERRER DETAILS
Patient telephone no:	Self / family / friends / education / work colleague

PRESENTING COMPLAINTS OR ISSUES
Mental Illness

CLINICAL NARRATIVE
History
Examination
Navigation team notes
Requests Wristband Adult
Referral(s) [redacted]

[redacted]

PROCEDURES	DIAGNOSES
Interventions Observation / cardiac monitor, pulse oximetry / head injury / trends **Near Patient Tests** None **Comments**	**Diagnosis** Depressive disorder, Suspected diagnosis **Comments** seen by psychiatric team with c/o feeling low in mood due to existing family issues. psych team impression nil acute psychosis noted. Discharge with safet netting advise

PLAN AND REQUESTED ACTIONS
Discharge Follow up: Community psychiatric support services **Comments to GP:** for gp follow up if symptoms persist

DISCHARGE DETAILS	
Date & time of discharge:	20 Jun 2021, 11:28
Discharge status:	Treatment complete
Discharge outcome:	Home

PERSON COMPLETING RECORD	SENIOR REVIEWING CLINICIAN
Name: [redacted]	**Name:**
Professional identifier:	**Professional identifier:**

CONTACT FOR FURTHER INFORMATION
If you require any further information regarding this patient's attendance, please contact the Nurse in Charge of ED on 01582 497062

The Luton and Dunstable University Hospital Emergency Department

Discharge Summary

Telephone No.: (01582) 491166

PATIENT DEMOGRAPHICS	GP PRACTICE
Patient Name: [redacted] **Date of birth:** 30 Jul 1975	**GP details:** [redacted]
Gender: [redacted]	ATTENDANCE DETAILS
NHS number: [redacted]	**Date & time of arrival:** 20 Jun 2021, 11:31
Other identifier: [redacted]	
Patient address: [redacted]	REFERRER DETAILS
Patient telephone no:	Self / family / friends / education / work colleague

PRESENTING COMPLAINTS OR ISSUES
Mental illness

CLINICAL NARRATIVE
History pt was seen directly by PLS team **Examination** **Navigation team notes** **Requests** Wristband Adult **Referral(s)** Mental Health Act assessment

[redacted]

PROCEDURES	DIAGNOSES
Interventions None **Near Patient Tests** None **Comments**	**Diagnosis** Direct admit to a specialty, Suspected diagnosis **Comments**

PLAN AND REQUESTED ACTIONS
Discharge Follow up: No referral **Comments to GP:** pt was seen by PLS, was admitted to EAU pending MHA assessment, further discharge letter to follow

DISCHARGE DETAILS	
Date & time of discharge:	23 Jun 2021, 08:19
Discharge status:	Treatment complete
Discharge outcome:	Ward - physical ward bed outside ED

PERSON COMPLETING RECORD	SENIOR REVIEWING CLINICIAN
Name: [redacted]	**Name:**
Professional identifier:	**Professional identifier:**

CONTACT FOR FURTHER INFORMATION
If you require any further information regarding this patient's attendance, please contact the *Nurse in Charge of ED on 01582 497062*

Next Appointment:
No Medical Appointment is booked, however, remains open to the secondary mental health service for Social Outcome Assessment

Summary of Appointment

I reviewed [redacted] in outpatient clinic over a telephone call on 26th August 2021, for the first time.

According to the available record, Mr Davies has been known to mental health services on and off since 2012, he often presents at a crisis point, threatening suicides though I gather that he has no history of serious self-harm. He was also found to be resourceful. He was last seen by

as One

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my predecessor on July 2021, it was concluded that he did not suffer from psychotic illness and that his main difficulties is related to emotional dysregulations, he was referred to the psychology department. He was offered DBT but was not willing to attend and he was subsequently discharged.

More recently on 19th July 2021, he was assessed in A&E when he had attended, for suicidal thoughts, he was then assessed by Abbs Hassan, CPN from CATT, who found him not meeting criteria for crisis intervention, he was given wellbeing service contact details and was discharged back to the GP.

During the phone call on 26th August, [redacted] said that he was in harry to go to a meeting as part of his work, he quickly mentioned that all he wanted was to live with a friend in a shared live scheme, he wanted us to help facilitating this. He reported that he has a lady friend that he has known for 10 years, she is not a girlfriend but a good friend, they have things in common and want to live together as part of "shared live scheme" he has already put his flat/ house up for sale and want to "move on" as soon as possible,

On enquiring about any symptoms, he described the following: I sometime get anxious, paranoid, stressed, obsessed with things, angry, depressed, elated, and sometimes normal

Due to time pressure and the fact that he wanted to leave asap, it was not possible to explore these beliefs all in the meeting. We agreed that I will review his case notes discuss his case with the team and call him back on Friday 3rd September 2021.

In terms of risks assessment- there were no suicidal, homicidal thoughts, no clear delusions or hallucination or active psychosis.

When I called him back on 3rd September 2021, Jamie sounded calmer, his speech was less pressurised. He stated that during our last meeting he was due to go for a meeting at the workplace, and he was "stressed" then.

I explained that his idea re shared live scheme was discussed with the MDT and it was agreed that he undergoes Social Outcome assessment (SOA) to identify his needs and to help him to further explore his options. He was disappointed that this will not happen soon but agreed that it was a positive step. He talked about some financial difficulties said he has already started to pay his friend privately to be his carer but was not sure if this was sustainable. I explained that he would have to wait for the SOA to take place, he could then, discuss his concerns regarding his social circumstances.

[redacted] also talked about his "paranoia" said this comes and goes and has been since the age of ? 16. He did not elaborate any further, however it was my impression that he was not experiencing any thoughts of persecution or abnormal beliefs or psychotic experiences at the time of the assessment.

He is currently not keen on talking therapy or medications.

Impression

Emotional instability and poor coping skills, it seems as though at times of stress [redacted] may become agitated and might present with quasi psychotic symptoms. However I gather that

he was tried on various antipsychotics and did not respond well to it in the past, today I suggested Promethazine PRN but he declined this too.

Plan agreed

No further outpatient appointment.

Awaiting SOA, to explore the option of shared lives scheme.

Actions for GP

None

Yours sincerely,

Electronically checked & approved by Doctor

Dr Nada Al-Asadi, MD, PGCert, MSc, MRCPsych,
Consultant Psychiatrist

[redacted]

NB: If you receive this letter in error, or it is not appropriate for your Practice please destroy any copies and ensure that the original is returned to sender. If you are on Docman, please use the Reject function, otherwise get in touch with the team using the above details.

[redacted]

[redacted]

Discharge from service

I am writing to confirm your discharge from the Adult Community Mental Health Services. As you are aware, you were re-referred to our service in July 2021 so that you could receive a Social Outcome Assessment. The outcome of this assessment was that you are not currently eligible for social care, as you did not have any identified eligible care and support needs under the Care Act 2014. As the Shared Lives scheme operates under the framework of adult social care, this unfortunately means that you would not be eligible for the scheme. On the positive, this is because you remain independent and capable of meeting your daily living needs, such as maintaining your home, work, nutrition and hygiene, to provide just a few examples. Following your assessment, you were referred to the Community Navigator service to explore community and I hope this has been of value to you.

I have discussed your care within the multi-disciplinary team to identify any other appropriate support. You had a psychiatric review with [redacted] (letter enclosed) who did not see any utility in further psychiatric care at this time. As your psychiatric review and Social Outcomes Assessment have been completed, we are discharging you from our service. You have previously accessed Mind, and continue to do so, along with your private therapist; we would encourage you to continue to appropriately utilise these resources. There are other services such as CALM (a wellbeing mobile phone app) and Saneline (0300 304 700) who provide support for those struggling.

Understandably, you remain concerned and distressed at not having contact with your daughter; however, as our team have previously discussed, we have no jurisdiction in this area and cannot provide meaningful support with this. We would strongly advise that you seek legal advice and/or advice from Children's Services, as this remains a legal issue, rather than a psychiatric one.

Whilst I appreciate you can feel at a loss at times and want to speak with someone, your recent contacts have not always been appropriate. As a community mental health service, duty is to be used for those in acute distress; whilst we would like to

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be a listening ear for individuals to vent their thoughts and feelings, this is unfortunately not possible. I am pleased to note that you remain settled in your mental health and a largely independent gentleman and, as such, we would ask that you seek support that is proportionate to your needs in the future.

Support Plan

1. Continue to engage with Mind and your private therapist to support you to manage your anxiety.
2. Engage with Community Navigators to identify community support services to support you to manage your anxiety.
3. We would advise you to contact Children's Services and other support services such as Fathers4Justice or Families Need Fathers to get support with gaining contact with your daughter.
4. Contact Relate via www.relate.org.uk for support managing your relationship.
5. Continue to engage with supportive activities such as going to the gym.
6. You have worked hard to identify stressors in your life and consider ways to reduce the stress; you discovered that origami and Tai-Chi were helpful for both distraction and relaxation purposes, and through MIND developed an interest in writing.

Risk Management Plan

1. Utilise community support services such as Saneline 0300 304 700, Night Light (open Friday-Tuesday, 01923 256391).
2. Utilise coping strategies developed during your work with [redacted] Wolfson and with your private therapist, for example: anxiety management strategies including a breathing technique (in for 5 and out for 7), as well as paired muscle relaxation (squeezing a muscle group as you inhale and relaxing it when you let your breath go). I also understand that you have benefited from intense exercise (quick burst of physical activity) or putting your face in cold water/an ice pack across your eyes and bridge of your nose, if you felt particularly overwhelmed with emotion.
3. If there is a deterioration in your mental health, contact your GP who can discuss appropriate support options and can re-refer to our service if required.
4. Contact 999 or attend A&E in an emergency.

I wish you all the best for the future.

Yours Sincerely,

[redacted]

[redacted]

[redacted]

[redacted]

Although you are currently still open to the team, our work together has now come to an end, so I wanted to write to you to summarise our journey. We started work on 18th October 2019 after your transfer of care from Welwyn City Adult Community Mental Health Service, and we met on an almost weekly basis until 6th August 2020 . During this time, I have seen a change in your presentation and the way that you think.

Initially the plan was to do a brief intervention piece of work for your anxiety, as you were experiencing some big changes in your life, including leaving your job. You were at

as one

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[redacted]

times so anxious that you would vomit; however I note that this has now stopped which I am glad about.

During our sessions I introduced you to CBT, and we looked at how thoughts, feelings and actions are connected; by changing one we can likely change the other aspects of the 'hot cross bun'. We spent some time looking and practicing thought challenging, as well as considering the emotions negative thoughts would evoke - in particular anxiety and anger. Although you were able to understand the concept, you sometimes found it difficult to come up with an alternative thought.

You worked hard completing the tasks and homework that was set, including noticing positive events in your life, and negative thinking styles and how to challenge that thought. You have worked hard to identify stressors in your life and consider ways to reduce the stress; you discovered that origami and Tai-Chi were helpful for both distraction and relaxation purposes, and through MIND developed an interest in writing. Together, I taught you some anxiety management strategies including a breathing technique (in for 5 and out for 7), as well as paired muscle relaxation (squeezing a muscle group as you inhale and relaxing it when you let your breath go). I also explained the benefits of intense exercise (quick burst of physical activity), or putting your face in cold water/ an ice pack across your eyes and bridge of your nose, if you felt particularly overwhelmed with emotion.
You have been resourceful in seeking help. Initially you would call the mental health help line on a regular basis, often just wanting to chat and be given reassurance for your anxiety. Together we looked at reasons that you felt compelled to call them, and later on in our work call me, and you noted that when you are anxious you find it difficult to think things through clearly and it helps to have other people's opinions. You also told me that you often turn to Lion Heart, who at one point had to put restrictions on your calls in place, as well as MIND and the Samaritans. During lock down you also sourced a private music therapist who you found helpful. We worked on putting a list of strategies you could implement to reduce your anxiety first, and then a list of other people who you could turn to for support. You reflected that your mum, dad and '_____ are all helpful people at times, as is your AA sponsor with whom you have a good relationship.

During our work you have had to put yourself in potential awkward or stressful situations, including job interviews and going to Bangladesh for the first time. You were able to manage your anxiety and alter your thoughts towards these experiences, and you found that both have been positive- you really enjoyed your trip at the start of the year and are now in full time employment.

I appreciate that at times your new job is stressful. This for a short while led to an increase in your calls and texts to me. However you did well to reduce these calls, and we considered other strategies for your anxiety, such as visualisation. Together we practiced visualising yourself in a safe and comforting place, imaging yourself being there using all of your senses. We also practiced visualising a potentially difficult future

[redacted]

[redacted]

scenario and how you could cope in it, imagining yourself walking through all the steps. With our practice scenario you were able to problem solve the various potential obstacles (buy dinner using cash so that you would get change and have cash for a parking meter in case it was needed the next day), which shows how far you have come in being able to take different perspectives and think through different angles of a situation.

Since moving to our team you have had your diagnosis changed from Paranoid Schizophrenia to Anxiety and potential complex PTSD. Although you had not agreed with the initial diagnosis, having it altered appeared to throw you a bit and perhaps impacted on how you saw yourself. I'd like to remind you that you are more than a diagnosis, and they are there to help inform a care plan and consider how we can best help you. As you know, we have therefore put you forward for a psychology assessment. In our last call you stated that you do not think you have a mental health problem and that everyone has ups and downs and goes through difficult things in life; whilst I agree that life often presents difficult situations, it is important to remember that you have a support network to help you through these times. Moreover, I know working on improving and maintaining your mental health is important to you, and you have worked hard over the years to do this; please remember that helping yourself may not be by using lots of services, and feeling obliged to attend a group or talk, but rather making sure you carve out time to do things that give you a sense of enjoyment, fulfilment or relaxation.

Going forward:

- You have a psychology assessment with [redacted]
- Should you feel the need to talk to our team you can contact duty on 01727 804674, or the Mental Health Help Line on 01438 843322
- You also receive support from AA, Lion Heart, MIND and Samaritans

I know that you have already written some of these strategies down, but as discussed, I would recommend writing a plan that includes which strategies to use and when for when you feel overwhelmed, stressed or anxious, so that you know what steps to follow to initially help yourself. I have included a plan for you to fill in if you wish. It has been a pleasure getting to know you, and I wish you the best of luck for the future. You have worked hard, and I am sure will continue to do so.

As a service we appreciate all feedback, so that we can continue to work towards always providing the best service possible. If you would like to leave feedback, you can do so here: https://www.hpft.nhs.uk/service-users/having-your-say/

Yours Sincerely,

[redacted]

as One

Our Values
Welcoming Kind Positive Respectful Professional

[redacted]

- What does it look like when anxiety increases?

- What do you do or not do?

- What might you say?

Signs I'm going off track are...

This might include:

as one

Our Values
Welcoming Kind Positive Respectful Professional

[redacted]

[redacted]

DISCHARGE FROM SERVICE

The above mentioned patient has been discharged as he was not wanting our services at this time. He did explain he was seeing yourselves for some physical ailments and was happy to continue to see you.He also will use the MHHL number if need be.

Therefore, I am writing to inform you that this patient will now be discharged from secondary care services.

Should you feel that this patient needs a re-referral please do not hesitate to do so.

Yours sincerely

[redacted]

as One

Our Values
Welcoming Kind Caring Respectful Professional

[redacted]

Good morning

Please see attached case note entry regarding your patient. Please arrange a review and refer to SPA if clinically indicated.

Many Thanks,

[redacted]

[redacted]

29/12/22 - screened, attach and cc to GP to request a review and formal referral if clinically indicated.
Ken Roberts

No Associated Referral

Preferred Name

Document

called, said that he doesn't want to live anymore. When I asked if he had thoughts of harming himself, he said he'll "trick himself and be sneak to accept things the way they are"

I asked again if he had thoughts of harming and if he can keep safe, his reponse was "It's a little more complicated than that" - I advised if he feels unable to keep himself safe or wants to harm himself to take himself to A&E or call 999.

Says he has a wife in London and a wife in Welwyn?

Says he is the "pop up man" and has been worn down.

I asked him if he wanted to make a referral or have a chat, he said he just wants a chat.

said he has a child who he hasn't seen for over 10 years. says that makes his life not worth living. I said I was sorry to hear that and he said I was insulting him

Says he is addicted to sex, overeating & alcohol.

Feels like he is "actually turning into his phone" - says he is now treading water, wants his daughter back for normal family life.
(Paul Zzgallacher, 29/12/2022 10.44, REFERRAL ADVISOR, NON CLINICAL)

URGENT
BG

Print Date: 29/12/2022

Page: 1

From: SPA3 (HERTFORDSHIRE PARTNERSHIP UNIVERSITY NHS FOUNDATION TRUST) <hpft.spa3@nhs.net>
Sent: 25 January 2023 08:18
To: DH, Sec (DAVENPORT HOUSE SURGERY) <sec.dh@nhs.net>
Subject: Jamie Davies - 428 422 6908

Dear Dr Dewji

Thank you for your referral, I have spoken to Mr Davies this morning, he has requested I discharge his referral and pass him back to your care. Please do not hesitate to refer again if it is clinically indicated.

Many Thanks,

[redacted]

[redacted]

[redacted]

Re: [redacted]

To: DH, Sec (DAVENPORT HOUSE SURGERY) <sec.dh@nhs.net>;NAGRA, Aman (HERTFORDSHIRE PARTNERSHIP UNIVERSITY NHS FOUNDATION TRUST) <aman.nagra@nhs.net>;FORBES-VASSELL, Naomi (HERTFORDSHIRE PARTNERSHIP UNIVERSITY NHS FOUNDATION TRUST) <n.forbes-vassell1@nhs.net>;SHIRI, Beauty (HERTFORDSHIRE PARTNERSHIP UNIVERSITY NHS FOUNDATION TRUST) <beauty.shiri@nhs.net>

[redacted]

I did not carry out an MSE, [redacted] called our Mental Health Helpline, (MHHL), number, which he does quite often. I advised him a referral has been made, and that one of my colleagues had tried to get hold of him yesterday, and that he was due for a triage call today. I also advised him to wait for the triage call before he made any decisions, but he said he didn't need to speak to a clinical member of staff, he was happy to continue to use the MHHL, and access support via his GP, and again asked for his referral to be discharged. There were no risks indicated, as stated in your referral, and at the time of the call he appeared to have capacity to make this decision.

Many Thanks,

Arthur Walsh
SPA Tele-coach

[redacted]

[redacted]

UC85

Universal Credit
Medical Report Form

Surname

Other Names

National Insurance Number

Date of Birth

Time Examination and Interview Started

Time Examination and Interview Ended

Time Report Completed

Date of Examination

Place of Examination

Examination Scheduled as

Healthcare Professional's Name

[redacted]

[redacted]

Client Interview

Medical Conditions and Treatment

Medical Conditions 1

Conditions Medically Identified
Mental Health Problem
Abdominal Problem
Pre-Diabetes
Upper Body Pain
Respiratory Problem
Minor Head Injury

Other Conditions Reported

Client states no other problems

Medication 2

Information given by client over the phone.

Sertraline (antidepressant) taken regularly every day at 100mg dosage once a day.

Side Effects Due to Medication 3

There are no reported side effects due to medication.

Description of Functional Ability 4

Having considered whether the condition is likely to vary during the average week and if the function can be carried out regularly and repeatedly taking into account, fluctuation, pain, fatigue, stiffness, breathlessness, balance problems etc, the description of functional ability is as follows:

Condition History

Mental Health Problem
Autistic Spectrum Disorder: Onset from birth.
He attended mainstream for both GCSE and A levels years.
He was given extra support with school work, with his reading and writing and obtained some qualifications.
He reports does not have much insight regarding this condition and does not feel affected or want to be affected by it.

Client has previous history Paranoid disorder and Paranoid Schizophrenia highlighted in the previous reports and the 113 but he insists this is misdiagnosis and he is not seeing any specialist for it and has no psychotic episodes and he is not on any antipsychotic medication.
He seemed reluctant to discuss this further, including any previous sectioning.

Anxiety and Depression: He claims was only diagnosed in January 2023 after his work ended, despite the evidence suggesting otherwise.
He feels he has continued to deteriorate further the past few months due to him not working and being anxious about his financial situation.
Symptoms: Low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks all the time, every day. GP is aware.
His panic attacks last about an hour, he struggles to go out due to this.
He is only forcing himself to leave the house when has an appointment with the Doctor as he knows it is important.
He has been seeing his GP more regularly recently due to deterioration. He is currently seeing the GP weekly.
He is also under the mental health team and the MIND service.
He has been having talking therapy daily and also has music therapy on the phone that he uses to help with his symptoms.
He is on antidepressant medications which was recently changed in January 2023.
He was on Fluoxetine 60mg once a day and now he is on Sertraline 100mg once a day.
He reports recent suicidal ideation in January 2023, he had thoughts of wanting to end it all which last for a couple of hours. His GP is aware.
His daughter is his protective factor.
He reports no attempts of suicide past or present.
He denies thoughts or attempts of self harm past or present.
He reports past problems with alcohol but he is 12 years sober. He was drinking self medicate as he felt lost. He attended AA services which helped him stop. He reports no relapse or cravings since stopping.
He reports has episodes where he becomes verbally aggressive towards other people but this is when provoked. This last happened a couple days ago at the pharmacy.

Abdominal Problem

Pre-Diabetes

Upper Body Pain

Respiratory Problem

Minor Head Injury
He confirms this was nothing major, he bumped his head last year but no problems due to this.

Social History

Evidence reviewed includes the Questionnaire, Med 3, 113, FRR4, previous report, GP Medical Record printout and UC Additional Typed Evidence, Call for Consent Form and POID2.

[redacted]

Telephone assessment. Attended alone.

Client lives alone in a house, on the ground floor.

Occupational History

Last worked in January as a chartered surveyor. His worked ended due to his mental health.

Not currently working or studying.

The client is right-handed.

Description of a Typical Day

Is stated that:

He goes to sleep late around 1am and wakes up at 08am, although his sleeps is broken up a lot at night due to his anxiety and worries bout his financial situation.
He has a partner who comes to see him every now and then when she is not working but will always call every day to prompt and support him, due his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.
Him and his partner are looking to move in together for further support due to his deterioration.
He is struggling to manage his day-to-day tasks like meal preparations so usually just eats cold food and snacks with prompting from his partner, due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks. He will have hot meals once or twice a weeks on the days his partner visits and cooks.
He is only getting washed and dressed once a week when he has to attend his appointments but otherwise will not do so unless prompted by his partner, due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.
He is not leaving the house at all except to attend his important appointments once a week, he is being prompted by his partner to get ready and go due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.
He misses being able to work, keep busy and have a stable income which he is not able to do currently due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.
His partner helps him keep up to date with his bills, unexpected mails and correspondence, due to his low mood, lack of motivation, poor concentration and agitation.
He regularly forgets to take his medication as he gets agitated and distracted easily, his partner always reminds him to take.
His partner supports him with housework and laundry and always does shopping for him, when she comes due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.
He worries without his daily support from his partner, he would self neglect due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.

Medical Examination Findings

The information contained in this section uses medical terminology and is intended for a reader with medical training. All relevant findings are explained in non-technical terminology in the appropriate sections earlier in the report.

General 30

Lower Limb 31

Upper Limb 32

Cardiac, Respiratory, Vascular 33

Vision, Speech, Hearing 34

Consciousness 35

Continence 36

Mental State 37

Behaviour
Coping at Interview : Moderate difficulty coping at interview
Arousal : Distracted
Rapport : Poor

Speech
Amount : Normal
Rate : Normal

[redacted]

[redacted]

Mental State 37

Volume : Normal
Content : Normal

Mood
Demeanour : Timid

Cognition - General
Orientation : Orientated in time, place and person
Prompting : Needed Prompting
General Memory : Adequate
Concentration : Poor

Insight
Insight : Good
Awareness of Danger : Adequate

Limited mental state examination due to telephone assessment.
Client reports recent suicidal ideation in January 2023 but with no plans or intent which GP is aware of.

Observed Behaviour 38

Lower Limb & Back

Upper Limb

Sensory

LCW and LCWRA Information 39

The client was found to meet the criteria for Limited Capability for Work and Work Related Activity

Telephone assessment.

Mr Davies has Mental Health Problem, Abdominal Problem, Pre-Diabetes, Upper Body Pain, Respiratory Problem and previous Minor Head Injury.

He has had Autistic Spectrum Disorder from birth although reports no symptoms, he was recently diagnosed with anxiety and depression in January 2023 when his work ended and he feels has continued to further deteriorate the past few months as he is not working and has been anxious about his financial situation. He describes daily symptoms of low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks. He is seeing the GP weekly for this and he is under the care of the mental health team and MIND service. He is on standard medication with recent changes, and he is having daily talking therapy.

[redacted]

His typical day shows, due his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks he is struggling with day-to-day tasks and has daily support from his and they plan on moving in together due to his deterioration. He is prompted and supported with personal care, medications and nutritional needs, due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks. He misses being able to go to work, he is not leaving the house at all except to attend his important appointments once a week, he is being prompted by his partner to get ready and go due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks. His partner helps him keep up to date with his bills, unexpected mails and correspondence and she is doing the housework, laundry and shopping, due to his low mood, lack of motivation, poor concentration and agitation.

On limited mental state examination, although spoke clearly he seemed distracted and had poor concentration, he required prompting. Therefore, based on the overall available evidence, a severe functional disability seems likely in reliably and repeatedly performing personal actions. He worries without his daily support from his partner, he would self neglect due to his low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks. His partner supports him with all

The available evidence suggests review in the medium term.

He is having weekly appointments with the GP, he has had recent medication change and is having input from mental health team and MIND service, his function could improve in 12 months.

[redacted]

23259060115410004002 7

Declaration

This form has been completed by a healthcare professional approved by the Secretary of State for Work and Pensions.

I have completed this form in accordance with the current guidance to UC examining healthcare professionals as issued by the Department for Work and Pensions.

I can confirm that there is no harmful information in the report other than indicated.

[redacted]

[redacted]

Harmful Information

Harmful Information - Not to be Copied to the Client 40

[redacted]

RESTRICTED - MEDICAL

[redacted]

[redacted]

[redacted]

15th August 2023

I refer to my PIP form sent to you on 21.3.23.

Section 2 of the Additional Information sent with the form referred to further medical evidence. My PIP was refused and I have now requested a MR.

I enclose medical records from my GP in support of my MR.

Please acknowledge receipt and confirm that these records will be provided to the Decision Maker in my MR.

Yours faithfully,

[redacted]

[redacted]

[redacted]

Personal Independence Payment 2,
Mail Handling Site A
[redacted]

17th August 2023

I refer to my PIP MR request. On 15.8.23 I sent GP records in support of my MR.

I rely on **AP v SSWP (PIP) [2016] UKUT 416 (AAC)** makes clear that a Tribunal (and, by implication, the DWP) should rely on the best medical evidence available and not simply rely on the HP's evidence. Judge Hemingway *stated '...this tribunal was required to say more than it did about the additional medical evidence and that its failure to do so did result in its either failing to have proper regard to relevant evidence or its failing adequately explain the conclusions it had reached in the light of the medical evidence.'*

You should properly consider my medical evidence.

I now enclose further evidence – WCA report, 22.5.23. I have been awarded LCWRA with effect from 13.2.23

You will see that at page 7 the HP:

- Details the problems that I have with day-to-day tasks including personal care, medications and nutritional needs due to 'low mood, lack of motivation, poor concentration, agitation, anxiety and panic attacks.'
- Says I am not leaving the house at all except to attend important appointments with prompting from my partner.
- Refers several times to the support needed from my partner.
- States 'a severe functional disability seems likely in reliably and repeatedly performing personal actions'.

I understand that PIP and UC are different benefits with different descriptors. However, the evidence of from the WCA report is relevant to PIP Daily Living Activities 1, 2, 3, 4, 5, 6 and 9 and Mobility Activity 1, all of which are covered in my PIP claim and Additional Information.

It is perverse and inconsistent for me to have been awarded LCWRA and not PIP when my conditions and how they affect me are very similar for the Descriptors for both benefits.

Please acknowledge receipt and confirm that this WCA report and my GP records will be provided to the Decision Maker in my MR. I trust that proper weight will be given to my medical evidence and the WCA report. If necessary I will refer the Appeal Tribunal to this letter.

Yours faithfully,

[redacted]